Thongpiny, Juba

The heat peaks from January to March, and cholera travels with the tanker water. Chlorinate the water where the trucks fill, before the dry season.

STATE	EXCESS HEAT DEATHS	WINDOW	DOMINANT HAZARD
Central Equatoria (state level)	490 forecast, 390 of them January–March; 390 already past (Jun–Sep)	6-month forecast, Oct 2026 – Mar 2027	Dry-season heat; cholera through tanker water; flash floods; food prices

15/15 recommendations checked. 10 corrected, 0 unverified, 5 contraindicated (the five "Do not do this" cards). All 9 For you steps were checked too, and 8 were corrected. Exposure figure: Climate Impact Lab El Niño Heat Mortality Tracker data, version of 26 October 2026, pre-peer-review, methods forthcoming. It is published for Central Equatoria state, not for Thongpiny or Juba, and it counts heat only. Thongpiny's share was not calculated, because no population figure for the neighbourhood was found. Cholera transmission had moved away from Juba by mid-2026 (UNICEF, June and July 2026), so the cholera actions here are readiness on a trigger, not an outbreak response. The funding target fell from the panel's SSP 649.9 million (US$112,500) to SSP 346.6 million (US$60,000) after verification (−47%): chlorination costs half what was assumed, single-dose vaccine campaigns are funded by the global stockpile, and the oral rehydration points moved to a trigger.

EXECUTIVE SUMMARY — AT A GLANCE



4.86°N 31.59°E, S. Sudan · Natural Earth

- **The risk.** 390 of Central Equatoria's 490 forecast heat deaths fall in January–March, when schools have twice closed after pupils collapsed.
- **For you.** From now on, boil or chlorinate every drum of tanker water, and keep an extra covered drum before January.*
- **The money.** SSP 86.7–688.6 million (US$15,000–119,200); chlorination at the filling points buys the most lives.
- **Do not.** Do not cap tanker water prices by decree: in 2024 the drivers struck and neighbourhoods went without water.

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* Suggested formulation: check with a health worker before sharing.

FOR YOU, AS A RESIDENT 9 STEPS

For a household in Thongpiny, El Niño's danger comes in two parts. The October–December rains are forecast wetter than usual, and they wash dirt into the river and the drains. Then the hottest months, January to March, arrive when water costs the most and iron-sheet rooms are hottest. The ranked actions below are written for the city and its partners. These are the ones you can take yourself.

Boil or chlorinate every drum of tanker water, keep drums covered, and store one extra covered drum before January *

PARTIALLY CONFIRMED FROM ACTIONS 1 AND 6

BY WHEN	COST TO YOU	PROTECTS
Now; the extra drum by 31 December 2026	SSP 2,500–5,000 or more per 200-litre drum (2024 prices; 2026 prices will be higher); fuel to boil, or chlorine (no 2026 price found)	the whole household, from cholera and from a dry-season price jump

Juba's two Urban Water Corporation filling stations treat their water, but about eight private riverside stations are untested. In 2023 most tankers were found filling at contaminated points. Chlorinating drinking water cuts diarrhoea by about a third (risk ratio 0.66 across 25 studies; Wolf et al., *Lancet* 2022). Ask the driver where he filled. In September 2024 the council's official price was SSP 2,500 a drum in town and SSP 3,500 in the suburbs, and the drivers struck. Many households paid SSP 5,000.*

Keep oral rehydration salts (ORS) at home, start them at the first watery stool, and know where to go *

PARTIALLY CONFIRMED FROM ACTION 2

BY WHEN	COST TO YOU	PROTECTS
By 15 November 2026	pharmacy price not found; treatment at the cholera centres is free	children and older adults with diarrhoea

Most cholera deaths are deaths from dehydration that ORS prevents: in pooled studies of young children, ORS cut diarrhoea deaths by 69% (Munos et al. 2010). Juba Teaching Hospital runs a cholera treatment centre of about 100 beds. If someone is vomiting, cannot drink, or is very weak, go there the same day. To report suspected cholera or ask where help is, call **6666**, the Ministry of Health and UNICEF line (UNICEF, June 2026).*

* Suggested formulation: check with a health worker before sharing.

From January, keep out of the sun from midday to 4 pm, and watch for heat collapse

PARTIALLY CONFIRMED · ROLE-SPECIFIC

BY WHEN	COST TO YOU	PROTECTS
January–March 2027	none	children, older people, outdoor workers

In February 2025 about 12 pupils a day collapsed in Juba, mostly in iron-sheet classrooms, and the Ministry of Health told people to stay indoors at peak hours. Under an iron roof the afternoon is the hottest time, so rest in the shade of a tree or a verandah, cool skin with a wet cloth, and drink water often. Confusion, hot dry skin, or fainting means heat stroke: cool the person and get help at once. Heat-day and school-closure notices go out on Eye Radio (98.6 FM) and Radio Miraya (101 FM).*

Choose one older, sick, or bedridden neighbour, and check on them every day from January to March

PARTIALLY CONFIRMED · FROM ACTION 5

BY WHEN	COST TO YOU	PROTECTS
Agree it by 15 December 2026	none	people who live alone or cannot leave bed

In the 1995 Chicago heatwave, being bedridden raised the risk of dying about eight times, and living alone about two times (Semenza et al., *NEJM* 1996). Juba is younger and hotter, so this evidence transfers only partly, but a daily visit costs nothing. Bring water, help them to shade, and look for the heat-stroke signs above.

Clear the drain in front of your plot before the rains end

PARTIALLY CONFIRMED · FROM ACTION 7

BY WHEN	COST TO YOU	PROTECTS
15 November 2026	none	your home, and the street from standing dirty water

On 12 August 2018 homes in Thongpiny, Kubri Albino, and Referendum were submerged, and residents blamed poor drainage. Plastic and rubbish in the drains push water into houses. ICPAC forecasts a wetter than usual October–December for southern South Sudan.

* Suggested formulation: check with a health worker before sharing.

Find out now whether your children's 2027 school year opens in February or April, and plan for heat days

PARTIALLY CONFIRMED | FROM ACTION 3

BY WHEN	COST TO YOU	PROTECTS
By 15 January 2027	a water bottle per child	schoolchildren

Schools closed for heat in March 2024 and again from 21 February 2025. In April 2026 the government's Governance Cluster endorsed an April–December school year, but it was not confirmed as law. If school opens in February, send every child with water, and plan who will mind them if schools close again.

Have every child under five measured with the arm band (MUAC) at the health centre before January *

PARTIALLY CONFIRMED | FROM ACTION 4

BY WHEN	COST TO YOU	PROTECTS
December 2026	transport; treatment programmes are free in principle	young children

A thin, malnourished child is far more likely to die of heat, fever, or diarrhoea. About 2.2 million children in South Sudan were projected to be acutely malnourished in 2026 (IPC, April 2026), and many nutrition sites closed after 2025's funding cuts. Juba still has outpatient treatment sites. Ask at your health centre which one serves Thongpiny.

Sleep under an insecticide-treated net every night, and keep the free one you were given

CONFIRMED | ROLE-SPECIFIC

BY WHEN	COST TO YOU	PROTECTS
Now, through the rains	none if you received one in the 2026 campaign	children and pregnant women, from malaria

Malaria causes about a third of clinic visits in South Sudan. In April 2026 the state health ministry and Health Link South Sudan began a free net distribution in all six counties of Central Equatoria, Juba included, and warned people not to sell them. If your household missed it, ask your health centre.

* Suggested formulation: check with a health worker before sharing.

Buy a month of dry staples during the December–January harvest, before lean-season prices climb

PARTIALLY CONFIRMED | ROLE-SPECIFIC

BY WHEN	COST TO YOU	PROTECTS
December 2026 – January 2027	no Juba figure; the national median basket was SSP 585,289 a month for six people (June 2026)	the household's food through March–June 2027

FEWS NET expects prices to ease from October to January with the second-season harvest, while staying far above the five-year average. In the bimodal south the lean season usually peaks before the June–July harvest. Store grain dry and off the floor, because pests are the main risk.

What to ask of the people in charge

- Ask Juba City Council when chlorination will be required at the private riverside filling stations, and when tankers will be registered to treated stations only (action 1).
- Ask the Ministry of General Education and Instruction whether the 2027 school year opens in April. If it opens in February, ask for a heat timetable that ends classes by 13:00 on forecast days of 40°C or more (action 3).
- Ask the Ministry of Health and the County Health Department what will open the oral rehydration points in Thongpiny if cholera returns, and whether a vaccine request will follow (action 2).
- If your household is on the Shabaka Meisha cash list, ask the Ministry of Agriculture and Food Security, through your quarter council, when the pending payments will reach Juba (action 8).

WHAT THIS PLACE SHOULD DO — RANKED

Ranked by expected lives saved per unit of effort before the peak. Backing lenses are shown per action; an action several lenses reached

* Suggested formulation: check with a health worker before sharing.

independently is the stronger bet.

1. Require chlorination at Juba's private riverside filling stations, and register tankers to treated stations, by 30 November

PARTIALLY CONFIRMED | PUBLIC HEALTH · SKEPTIC · HISTORIAN · EMERGENCY MANAGER · FRONTLINE · PHILANTHROPIST

ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
Juba City Council, with the Urban Water Corporation; Red Cross for dispensers	30 November 2026	SSP 86.7 million (US$15,000) for chlorine and dispensers; the rule itself costs nothing	cholera and diarrhoea deaths, all year

Most of Juba buys water from private tankers. The Urban Water Corporation's two stations treat their water; about eight private riverside stations are untested, and no chlorination rule is enforced. Chlorinating drinking water reduces diarrhoea by about a third (risk ratio 0.66, 95% CI 0.56–0.77, 25 studies; Wolf et al., *Lancet* 2022, peer-reviewed). Chlorine dispensers at water points were associated with 24–30% lower odds of death in children under five, at about US$40 per disability-adjusted life year (Kremer et al., NBER working paper w30835, not peer-reviewed). Authorities have acted on this before: in July 2016 police were posted on the Nile bank to stop tankers drawing raw river water, but the rule did not last.

* Suggested formulation: check with a health worker before sharing.

Transferability: the mortality evidence comes from rural water-point trials in Kenya and elsewhere, not from tanker supply chains. Dosing at a filling station is a different intervention with no published cost or effect, so the cost here is an estimate from dispenser prices.

2. Agree a cholera trigger now: oral rehydration points in Thongpiny and the hotspots, surge supplies at Juba Teaching Hospital, and a vaccine request, all released when cases return

PARTIALLY CONFIRMED | EMERGENCY MANAGER · PUBLIC HEALTH · PHILANTHROPIST · HISTORIAN · FRONTLINE | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
Ministry of Health and County Health Department, with WHO, UNICEF, and the Red Cross	Agreed by 30 November 2026; released on the trigger	SSP 144.4 million (US$25,000) hospital contingency; SSP 173.3 million (US$30,000) for 10 rehydration points if triggered	deaths from late care, as when ~50 patients arrived overnight in April 2025

South Sudan's 2024–26 outbreak is its worst since independence: 109,908 cases and 1,720 deaths by 31 July 2026, a case fatality rate of about 1.6%, against WHO's target of under 1%. Thongpiny ran its own treatment centre in 2014. By mid-2026 transmission had moved to Unity, Upper Nile, and Jonglei, so the right step in Juba is readiness, not a standing operation. The trigger should be confirmed cases in Juba County on the Ministry of Health and WHO weekly line list. Oral rehydration solution cuts diarrhoea deaths in young children by 69% (Munos et al. 2010). A single dose of oral cholera vaccine was 87.3% effective in Juba in 2015 (adjusted; Azman et al., *Lancet Global Health* 2016, small sample). Reactive campaigns now use one dose, and the global emergency stockpile was below its target in 2026, so a request is only granted if transmission is confirmed.

* Suggested formulation: check with a health worker before sharing.

Transferability: the vaccine evidence is from Juba itself. The rehydration evidence is from South Asia. The cost of SSP 17.3 million (US$3,000) per rehydration point is an unverified estimate; no published cost was found.

3. Confirm whether the 2027 school year opens in April; if not, publish a heat timetable that ends classes by 13:00 on forecast days of 40°C or more

PARTIALLY CONFIRMED | EMERGENCY MANAGER · SKEPTIC · HISTORIAN · ECONOMIST · PUBLIC HEALTH | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
Ministry of General Education and Instruction, with the Ministry of Health and the Meteorological Department	15 January 2027	none for the decision; drinking water at schools not costed	pupils collapsing in iron-sheet classrooms, and weeks of lost school

Schools closed nationally for heat in March 2024 (forecast 41–45°C) and from 21 February 2025, reopening on 17 March, after about 12 pupils a day collapsed in Juba. The ministry said in 2025 that "February is not the right season." In April 2026 the Governance Cluster endorsed an April–December school year by amending the General Education Act, but no record shows parliament passed it. If the 2027 year opens in February, a timetable announced in advance, with water at every school, keeps children learning and out of the afternoon heat.

* Suggested formulation: check with a health worker before sharing.

Transferability: no trial anywhere shows that school heat timetables reduce deaths. This rests on the collapses recorded in Juba and on common sense, not on an effect size.

4. Screen children under five with the arm band and treat severe malnutrition before the January–March heat

PARTIALLY CONFIRMED | PUBLIC HEALTH · PHILANTHROPIST | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
A nutrition-cluster partner running outpatient treatment in Juba, accountable to the Ministry of Health	December 2026 – March 2027	SSP 115.5 million (US$20,000), about 55 children at SSP 2.07 million (US$358) each, all-in	deaths of wasted children, whom heat, fever, and diarrhoea kill first

IPC projected about 2.2 million acutely malnourished children nationally in 2026 (April 2026 update). The 2025 nutrition appeal was about 27% funded, hundreds of sites closed, and treatment coverage fell from 37% to 24%. Juba still has outpatient sites, and admissions at Al-Sabbah children's hospital rose in 2025. Ethiopia's national community-based programme cost US$762 per death averted (Laillou et al., *Maternal & Child Nutrition* 2022).

Transferability: the cost per death is a national average from rural Ethiopia; the extra cost in urban Juba will be higher. Route the money through a partner that runs a site, not through the County Health Department, which has no record of disbursing at this scale.

5. Quarter councils, churches, and mosques list people who live alone or are bedridden by mid-December, and pair each with a neighbour for daily checks

PARTIALLY CONFIRMED | FRONTLINE | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
Thongpiny quarter council and boma chiefs, with faith groups	Lists by 15 December 2026; visits January–March 2027	volunteer time; unfunded	unseen deaths of the isolated during heat days

* Suggested formulation: check with a health worker before sharing.

Pair this with short Juba Arabic radio spots on Eye Radio and Radio Miraya on heat-collapse and cholera signs. People in Thongpiny remember July 2016, when the UN base there sheltered more than 4,000 people and a shell struck a trailer of displaced people. A warning carries more weight from the quarter chief or pastor than from an official broadcast. Being bedridden raised the odds of a heat death about eightfold in Chicago in 1995 (Semenza et al., *NEJM* 1996, a case-control study).

Transferability: weak. The evidence is from older adults in a temperate city. Juba's population is young, and its heat deaths are likelier among infants and outdoor workers. The action is kept because it is free, not because its effect is known.

6. Agree a written dry-season water price with the tanker operators by 30 November, instead of setting one by decree

PARTIALLY CONFIRMED | EMERGENCY MANAGER · SKEPTIC | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
Juba City Council, with the tanker operators and the Urban Water Corporation	30 November 2026	none	a strike that cuts water at the peak of the heat and sends people to the river

In September 2024 the council halved the price to SSP 2,500 a drum; drivers refused, neighbourhoods went without water, and a uniform SSP 4,000 was agreed around October 2024. The council has negotiated before (March and October 2024), so an agreed price is within its power. Tying the price to fuel cost has no precedent here, but diesel in Juba more than doubled between February and August 2026, so any fixed price will break.

* Suggested formulation: check with a health worker before sharing.

Transferability: this is local precedent. The most recent drum price found (about SSP 10,000 per 250 litres, 2025) comes from a single open letter and is weak evidence.

7. Clear Thongpiny's drains and culverts before the October–December rains end

PARTIALLY CONFIRMED | EMERGENCY MANAGER · FRONTLINE · ECONOMIST | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
Juba City Council, with community work days	15 November 2026	not costed	flooded homes and standing contaminated water

On 12 August 2018 homes in Thongpiny were submerged, and residents blamed poor drainage. ICPAC forecasts wetter than usual rains for southern South Sudan this October–December (reported August 2026). In April 2025, three days of rain were followed by about 50 cholera admissions in one night at Juba Teaching Hospital.

Transferability: local precedent, but no study measures the effect of drain clearing on deaths. A claim that run-off from Jebel and Munuki drains toward the airport beside Thongpiny could not be verified.

8. Pay the pending Shabaka Meisha cash transfers to Juba households before January, and ask the payment agent about a channel that does not need bank notes

PARTIALLY CONFIRMED | ECONOMIST | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
Ministry of Agriculture and Food Security, with UNOPS (payment agent) and the World Bank	31 December 2026	none new: the money is already committed	households cutting water and food at the worst time

The World Bank-financed safety net (US$199 million over five years, 157,500 households in 20 counties) made its first payments in Juba County in December 2024. On 4 August 2026 the ministry said about six payment cycles were pending because of a local-currency shortage. The binding constraint is bank notes, not money, so a non-cash channel would release committed funds.

* Suggested formulation: check with a health worker before sharing.

Transferability: the arrears are confirmed, but the ministry's statement does not name Juba, give an amount per household, or mention mobile money. Whether mobile money is permitted could not be verified.

9. Review Juba's humanitarian cash transfer value every month, not periodically, while the pound is falling

PARTIALLY CONFIRMED | FOOD SUPPLY CHAIN · ECONOMIST | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
The Cash Working Group, with WFP and REACH	From the November 2026 round	a monthly review costs nothing; top-ups cost whatever the gap is	cash aid losing a third of its value between reviews

The adjustment rule already exists: transfer value equals the survival basket plus a 15% buffer. Juba ran 19% over its reference in May 2026 and 37% over in June before the value was reset. The national median food basket rose 22% in June. Trigger: a Juba basket rise of 15% or more in a month on the REACH market monitor (monthly, about three months late), cross-checked against WFP's weekly market monitor.

Transferability: local. As first proposed ("create an indexation rule") this was redundant; the rule exists, and the gap is how often it is applied.

10. Hold to the 10 May 2026 Nimule road deal so food trucks keep moving through the lean months

PARTIALLY CONFIRMED | FOOD SUPPLY CHAIN | ADAPTATION INVENTORY: NOT CHECKED

OWNER	BY WHEN	COST	PREVENTS
South Sudan Revenue Authority and the Economic Reform Committee, with the joint task force	Now to March 2027	none	a repeat of the two-week Elegu blockade

After an attack on 28 April 2026, truckers blockaded the Elegu crossing. The deal signed on 10 May cancelled the SSP 90,000 exit stamp duty, checkpoint collections at Nesitu and elsewhere, and illegal immigration and police fees. On 30 July 2026 the Economic Reform Committee ordered more than 4,000 trucks held at Nesitu released, so compliance is already contested. Juba's maize comes overland from Uganda.

* Suggested formulation: check with a health worker before sharing.

Transferability: local. The US$70 driver registration fee was not part of the deal; it was kept as lawful in December 2024.

* Suggested formulation: check with a health worker before sharing.

WHO DOES WHAT

RACI CHART

R = Responsible (does the work) · **A** = Accountable (one per action; signs off and answers for it) · **C** = Consulted (asked before) · **I** = Informed (told after). Built from the owners named in the verified actions. No one here has been contacted or has agreed to a role; every row is a proposal.

Action	Juba City Council	Urban Water Corp.	Ministry of Health & County Health	WHO, UNICEF & WASH cluster	South Sudan Red Cross	Ministry of Education	Quarter council & faith groups	Nutrition partner	Agriculture ministry & UNOPS	Cash Working Group	Revenue Authority & Reform Committee
1. Chlorination at filling stations	**A**	**R**	**C**	**C**	**R**		**I**				
2. Cholera trigger and readiness	**I**		**A/R**	**R**	**R**		**I**				
3. School calendar or heat timetable	**I**		**C**			**A/R**	**I**				
4. Arm-band screening and treatment			**A**	**C**			**I**	**R**			
5. Neighbour checks and radio			**I**		**C**		**A/R**				
6. Agreed water price	**A/R**	**C**					**I**				
7. Drain clearing	**A/R**						**R**				
8. Safety-net arrears							**I**		**A/R**	**I**	
9. Monthly cash review									**I**	**A/R**	
10. Nimule road deal										**I**	**A/R**

What the chart shows: Juba City Council is accountable for three rows (1, 6, 7), and none of the three needs new money, only a decision. The Ministry of Health carries the two health rows. Half the rows (3, 6, 8, 9, 10) are decisions or schedules rather than spending.

WHERE MONEY DOES THE MOST

FUNDING

The Philanthropist lens, reconciled against the ranked actions and verified. Placements are ranked by marginal lives saved per dollar. The envelope is bounded by what this place can spend before the peak, not by what it needs. It covers Thongpiny and the neighbouring quarters served by the same filling stations and health facilities.

FLOOR

SSP 86.7 million (US$15,000)

TARGET

SSP 346.6 million (US$60,000)

CEILING

SSP 688.6 million (US$119,200)

LARGEST UNCERTAINTY

Whether cholera returns to Juba this season

Floor = chlorination, SSP 86.7 million: 5,000 households (an assumed catchment, as no population for Thongpiny was found) × SSP 3,466 (US$0.60) a month × 5 months, from Evidence Action's figure of under US$1.50 per person per year. Target = floor + severe malnutrition treatment (SSP 115.5 million: about 55 children at SSP 2.07 million (US$358) each, a South Sudan NGO costing from 2018) + a hospital cholera

* Suggested formulation: check with a health worker before sharing.

contingency (SSP 144.4 million, an assumed amount, released on a trigger) = SSP 346.6 million. Ceiling = target + 10 rehydration points (SSP 173.3 million, SSP 17.3 million (US$3,000) each, unverified, released on a trigger) + single-dose vaccine running costs for 40,000 people (SSP 168.7 million: 40,000 × SSP 4,217 (US$0.73) per dose, from Juba's 2015 campaign costs, only if the Ministry asks and the global stockpile's funding does not cover it) = SSP 688.6 million. The target moved −47% in verification.

RECIPIENT	USE	AMOUNT	MECHANISM	BASIS	VERDICT
South Sudan Red Cross, Juba branch, with a WASH-cluster partner	Action 1: chlorine and dispensers at water and filling points, November–March	SSP 86.7 million (US$15,000)	direct grant	About US$40 per DALY, under-5 deaths −24–30% (Kremer et al., working paper; rural Kenya and pooled trials)	Partially confirmed
Nutrition-cluster partner running outpatient treatment in Juba	Action 4: screening and treatment running costs	SSP 115.5 million (US$20,000)	direct grant	US$762 per death averted (Ethiopia, Laillou 2022); US$358 per child all-in (South Sudan, 2018)	Partially confirmed
Juba Teaching Hospital cholera centre, through WHO or the Ministry of Health	Action 2: surge supplies	SSP 144.4 million (US$25,000)	pre-positioned stock, released on a trigger	No unit cost; pays off only if an outbreak returns	Partially confirmed
South Sudan Red Cross volunteers	Action 2: 10 oral rehydration points (ceiling only)	SSP 173.3 million (US$30,000)	released on a trigger	No published cost per point or per death; ORS −69% diarrhoea deaths (Munos 2010)	Unverified cost
Ministry of Health, Central Equatoria	Action 2: single-dose vaccine running costs (ceiling only)	SSP 168.7 million (US$29,200)	top-up, only on request	US$0.63–0.73 per dose delivered (Juba 2015); likely already covered by the global stockpile's funding	Partially confirmed

A funder's view: Juba is left out because it is the capital. The December 2024 cholera allocation from the South Sudan Humanitarian Fund went to six other hotspots, and the Red Cross flood protocol that names Juba County fires only on a June–September forecast of the Sudd's extent.

* Suggested formulation: check with a health worker before sharing.

Nothing pre-arranged covers cholera or heat here. The cheapest life saved is chlorine bought this month, not beds bought in February.

Do not do this

CONTRAINDICATED · ADVICE THAT WOULD COST LIVES HERE

Advice that circulates for this hazard and would cost lives here.

Do not cap tanker water prices by decree

CONTRAINDICATED

In September 2024 Juba City Council halved the price to SSP 2,500 a drum. The drivers struck, some neighbourhoods got no water, and households turned to the river. An earlier threat of an SSP 100,000 fine (2023) was also ignored. A cap at the height of the heat cuts supply exactly when people need it most.

Instead: agree a written price with the operators before the dry season (action 6), and chlorinate at the filling point (action 1).

Do not buy or drink water from tankers filled at the river bank, untreated

CONTRAINDICATED

Juba's private riverside filling stations are untested, and in 2023 most tankers were found collecting from contaminated points. Untreated tanker water has been named as the route of cholera in every Juba outbreak on record, from 2014 to 2025.

* Suggested formulation: check with a health worker before sharing.

Instead: ask where the truck filled, and boil or chlorinate every drum (For you, step 1).*

Do not close every school for two weeks with no plan for where the children go

CONTRAINDICATED

The 2024 and 2025 heat closures ran two to three weeks with no contingency plan, and Integrity South Sudan publicly criticised the 2025 one. Children sent home still face the heat, often under iron roofs or fetching water, and lose school in a country where most children are already out of it.

Instead: move the school year (April opening) or announce a 13:00 heat timetable in advance, with water at every school (action 3).

Do not give heat advice that assumes fans, air conditioning, cooling centres, or piped water

CONTRAINDICATED

Juba's classrooms and most homes have iron-sheet roofs and no reliable electricity, and only a small share of households have piped water. Advice copied from cities with grid power tells people to do what they cannot, and leaves out what they can.

Instead: shade, timing (out of the sun from midday to 4 pm), wet-cloth cooling, stored treated water, and the signs of heat stroke (For you, step 3).*

Do not count on pre-arranged money to reach Juba for cholera or heat this season

CONTRAINDICATED

The South Sudan Red Cross flood protocol (approved October 2025) does name Juba County, but its trigger is a forecast rise in the Sudd's flood extent, checked from 1 June to 30 September, so it cannot fire before this peak. No anticipatory fund, insurance pool, or parametric heat cover for South Sudan covers cholera or heat. Planning that waits for this money will wait past March.

Instead: fund readiness now from discretionary money (see the funding section), and agree the cholera trigger in action 2.

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
2014 (May)	Cholera outbreak; Tongping (Thongpiny) ran its own treatment centre and was among Juba's most affected areas	6,269 cases and 156 deaths nationally	UNICEF chlorinated tanker loads; no lasting filling-station rule
2015 (June)	Cholera in Juba and Bor during a strong El Niño; the Ministry named untreated tanker water as a risk	705 cases and 32 deaths by 5 July	MSF gave a single vaccine dose to about 160,000 people in Juba; 87.3% effective
2016 (July)	Police posted on the Nile bank to stop tankers drawing raw river water	—	Not sustained
2018 (12 August)	Homes submerged in Thongpiny, Kubri Albino, and Referendum after heavy rain	No deaths reported	Nothing recorded
2024 (March)	Schools closed nationally for heat, forecast 41–45°C, at the tail of the 2023–24 El Niño	No verified count	Closure repeated in 2025
2024 (September)	Council halved the tanker water price; drivers struck	Neighbourhoods without water	A uniform SSP 4,000 a drum agreed around October 2024
2024–26	South Sudan's worst cholera outbreak since independence; Juba County 2,448 cases by 17 January 2025	Juba County 30 deaths by January 2025; 1,720 deaths nationally by July 2026	700,000 vaccine doses earmarked for Juba (January 2025)
2025 (21 February)	Schools closed after about 12 pupils a day collapsed in Juba, mostly in iron-sheet classrooms	No verified count	The ministry moved toward an April–December school year (endorsed April 2026)

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
2025 (April)	After three days of rain, about 50 cholera patients arrived at Juba Teaching Hospital in one night	Hospital "beyond its capacity"	WHO kits for 100 patients a week
2026 (April–May)	Truckers' blockade at Elegu after an attack on 28 April	Food supply cut for about two weeks	Deal of 10 May cancelled checkpoint charges; compliance contested by July

The recurrence nobody has noticed: regulating where tankers fill has been promised in every cholera outbreak since 2014 and never built, and cholera keeps returning to the same quarters (Gumbo, Munuki, Gudele, Tongping). A second lag is also worth knowing. If this season's rains raise Lake Victoria as in 1997, the high water reaches Juba's river islands (Gondokoro, Mangalla) a year or more later, in 2027–28, long after attention has moved on.

WHERE THE EXPERTS DISAGREE

UNRESOLVED

- **El Niño will bring wetter rains and floods to Juba** versus **El Niño is the wrong frame for Juba**. The Economist and Philanthropist lean on ICPAC's wetter than usual October–December forecast for southern South Sudan. The Skeptic notes that rainfall here peaks in July–August, that ICPAC's 2023 forecast left central South Sudan dry, and that the heat comes every February–March regardless. A study of El Niño's rainfall correlation at Juba would settle it; none was found.
- **Move the school holiday into late February–March** versus **keep school open on a heat timetable**. The Economist backs the first, and the government's endorsed April–December calendar points the same way. The Emergency Manager, Historian, and Skeptic prefer a timetable, so children are not sent into hotter homes and water queues. Whether parliament passes the calendar change settles it for 2027.
- **Cash beats food aid** versus **cash fails when there are no bank notes**. More than 90% of people surveyed preferred cash (DanChurchAid, 2017), but the banknote shortage lasted through August 2026 and the pound fell about 12% in a month. Cash or vouchers stay the default while markets work, with monthly re-indexing (action 9) and food as a fallback.

* Suggested formulation: check with a health worker before sharing.

WHAT NOBODY COVERED

BLIND SPOT

Outdoor workers. The tracker's heat deaths will fall partly on boda-boda riders, market vendors, water carriers, and construction labourers working through the January–March afternoons. No lens found any data on them in Juba, and no action here protects their working hours or income. Heat deaths here are also unlikely to be recorded as heat: they will be coded as malaria, diarrhoea, or fever, so nobody will see whether any of this worked unless someone counts.

HOW TO READ THIS

METHOD

The panel is author-built. Eight simulated expert lenses were used to generate these recommendations. Where they agree, that is a strong hypothesis, not a consensus of the field. Every recommendation was then independently checked for evidence, transferability to this location, and feasibility before the peak; the verdict tags carry the result.

Mortality figures are resolved at state level (Central Equatoria), not at neighbourhood scale. The El Niño projection covers *heat only*; cholera, flood, malnutrition, and food prices are additional to it and are carried here by the historical record rather than by a published number.

HOW THIS BRIEFING WAS PRODUCED

METHODOLOGY

- **Scope.** The location was given as "Thongpinay, Juba, South Sudan" and built as Thongpiny (also written Tongping or Hai Tongping), a neighbourhood of Juba city, about 1.5 km southwest of Juba International Airport. Its payam could not be confirmed. Climate Impact Lab figures exist only for Central Equatoria state (GADM SSD.1_1); sub-city detail is used for hazard exposure and action design, not for death counts.
- **Exposure.** Climate Impact Lab El Niño Heat Mortality Tracker data, file `v261026_adm1_output_BASE_rounded_cust.csv` (version of 26 October 2026, pre-peer-review): Central Equatoria, 390 additional heat deaths June–September 2026, 490 forecast October 2026 – March 2027 (October 33, November 33, December 38, January 120, February 130, March 140), 880 in total; South Sudan, 12,500; the world, 857,400. This update supersedes the September report *451,000 at Risk*. Method: ECMWF SEAS5 seasonal forecasts applied to temperature–mortality relationships against a 1996–2025 baseline. The figures count heat only; we inferred this from the data, because cold countries show exactly zero in every winter month. They are never added to the Impact Map's long-run projections.
- **Expert panel.** Eight simulated expert lenses (emergency manager, public health scientist, frontline responder, skeptic, economist, historian, philanthropist, and food supply chain analyst) each researched this place independently and in parallel, from web sources, to a fixed brief.

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- **Reconciliation.** Duplicate actions were merged and backing lenses recorded. Convergence was tested for shared bias (whether the evidence behind an agreed action came from a setting like this one) before it counted in an action's favour. Contradictions were kept, not resolved by dropping a side. Actions were ranked by expected lives saved per unit of effort before the peak, each was given a named payer and trigger or marked unfunded, and the funding placements were reconciled against the ranked list. The panel's vaccine campaign was folded into the cholera trigger (action 2) after verification found Juba no longer a named hotspot.
- **Verification.** Six verifier agents checked every recommendation on three tests: evidence (the primary source and its effect size), transferability (whether that evidence came from somewhere like here), and feasibility (whether it can be done here before the peak). Financial actions were also checked for availability in this jurisdiction, a named payer, a trigger threshold, and whether they already exist. Food actions were checked for whether their price series are actually published and when. The funding envelope was checked for real recipients, published unit costs, and duplication, then re-summed.
- **Who does what.** The RACI chart is built from the owners named in the verified actions, with one accountable party per action. It is a proposal: no organisation named in it has been contacted or has agreed to a role.
- **Verdicts.** *Confirmed*: evidence, transferability, and feasibility all hold. *Partially confirmed*: holds after the corrections shown. *Unverified*: could not be checked to a primary source. *Contraindicated*: would do harm or waste the window here.
- **Adaptation Inventory.** Not consulted in this run. The inventory loads its data in the browser, and verifiers could not read it. No action here should be read as having no precedent; none was checked.
- **Money.** Costs are shown in South Sudanese pounds (SSP) with US dollars in parentheses, at SSP 5,777 = US$1, the mid of the Bank of South Sudan's buying (5,690.49) and selling (5,863.81) rates shown on its homepage, read on 30 October 2026. The bank does not date the rate. The parallel market was about SSP 6,600 in late June 2026 (FEWS NET), so dollars buy more in the market than this rate shows.
- **Reader.** No role was given, so the For you section is written for a resident. Its steps were taken from the verified actions and from what the panel found about households here. Any step not drawn from a ranked action was verified on the same three tests. The rest of the report is written for the city and its partners.
- **Language.** This briefing is in English only. English is South Sudan's official language and the working language of its government. The everyday language of Juba is Juba Arabic, which has no settled written standard, so a translation could not be made to a standard a local reader would trust.
- **Health wording.** Passages where a wording error could hurt someone (water treatment, oral rehydration salts, danger signs, heat, and malnutrition checks for children) are marked "* Suggested formulation: check with a health worker before sharing." They were checked

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against published sources, not by a health professional. They are suggestions, not medical advice, and anyone passing them on should confirm them with a doctor, pharmacist, nurse, or community health worker first. The marker was added on 2 November 2026.

- **Rules.** No cost is derived by dividing a published total, and no local figure is invented; where data is thin, the report says so and shows the containing geography's number with its arithmetic. Every source is listed below with its verification status.
- **Limits.** The panel and verifiers are AI research agents working from web-available sources, not human experts, and no one in this place was interviewed. Many Eye Radio pages carry 2026 dates that are crawl dates on older articles; verifiers re-dated every one they used, and one precedent (police on the Nile bank) moved from 2026 to 2016. No 2025–26 price was found for a drum of water, chlorine tablets, or ORS in Juba. No population figure for Thongpiny, no Juba cholera data for July–October 2026, and no 2026 IPC phase for Juba County were found. The locator map shows Thongpiny at country scale.

References · status after verification

Climate Impact Lab, El Niño Heat Mortality Tracker data v261026 (downloaded 30 Oct 2026); tracker · confirmed

Water and cholera: Wolf 2022; Kremer, NBER w30835; Evidence Action dispensers; Juba filling stations; police on the Nile bank, 2016; tanker ban, 2023; price cap and strike, 2024; Kapuri, May 2024; Urban Water Corporation price; open letter, 2025 drum price · partially confirmed

Cholera care: UNICEF SitRep 6, July 2026; UNICEF SitRep 5, June 2026 (hotline 6666); WHO, Juba Teaching Hospital centre; 50 admissions in a night; Munos 2010; Azman 2016; Juba vaccine campaign costs; MSF, Juba 2015; vaccine stockpile; cholera SitRep 64; 2014 outbreak; 2015 outbreak; IFRC rehydration point guidelines · partially confirmed

Heat and schools: closure, Feb 2025; closure extended; closure, Mar 2024; April–December calendar; Education Act amendment; Integrity South Sudan criticism; Semenza 1996; health ministry advice · partially confirmed (Save the Children page returned 403: unverified)

Nutrition and malaria: IPC update, April 2026; Laillou 2022; WHO nutrition services; nutrition funding cuts; net campaign, April 2026; Health Cluster bulletin · partially confirmed

Community: HRW, July 2016; Thongpiny flood, 2018; Thongpiny location; Eye Radio; Radio Miraya; ICPAC outlook, reported Aug 2026 · partially confirmed

Money: Bank of South Sudan rate; Shabaka Meisha launch; payment delays, Aug 2026; Red Cross flood protocol; WFP drought trigger, July 2026; SSHF cholera allocation, Dec 2024; OCHA update, July 2026 · partially confirmed

Food: REACH JMMI, June 2026; REACH JMMI, July 2026; FAO GIEWS South Sudan; FEWS NET outlook, June 2026; Nimule road deal; Nesitu trucks, July 2026; cash preference; banknote shortage; WFP rations, Oct 2025 · partially confirmed

Exposure data · Climate Impact Lab, 451,000 at Risk · Impact Map · Adaptation Inventory

Generated 30 October 2026 for Thongpiny, Juba. El Niño Ready is volunteer-run and is not affiliated with Climate Week NYC, The Climate Group, or Columbia University.

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